Weekly paclitaxel + trastuzumab → trastuzumab continuation

National Cancer Control Programme (NCCP) 00815 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects intravenous (IV) trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

STEP 1

Before the first visit

Blood tests, medicines review and the consent discussion come first. Confirm baseline and planned ongoing cardiac assessment.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Paclitaxel is an IV infusion. Trastuzumab may be IV or, if locally approved and prescribed, a subcutaneous injection. Staff confirm formulation, observation and access needs at every phase change.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

Sources: NCCP 00815 v3 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.